

FINAL Wound Review

Tuesday, 3 February 2026 5:18 pm

F2F

^presents with ^

^New to me, presents for wound review

^Planned present for ^

Subjective

- Progress: ^Improving / Stalled / Deteriorating
- Pain: ^Score /10, ^Constant / During dressing changes only, ^Overall Improving
- Interim events: <Fevers / Systemic symptoms / Antibiotics completed?>

Objective

Wound Assessment

- Location: ^Site
- Dimensions: (Length) x (Width) x (Depth) in mm
- Tissue Type (TIME):
 - T (Tissue): ^% Granulation / % Slough / % Eschar / % Epithelial. Colour of the tissue bed.
 - I (Infection): ^No/Signs of clinical infection? Odour / Heat / Swelling / Erythema
 - M (Moisture): ^Exudate: Low / Med / High ^Serous / Haemoserous / Purulent. Is there any bleeding?
 - E (Edge): ^Advancing / Stalled / Rolled / Macerated
- Periwound: ^Healthy / Eczematous / Cellulitic / Macerated
- Circulation: ^Pulses present? / Capillary refill time:

Clinical Photography:

Consent obtained for clinical photography; image uploaded to gallery/record.

Assessment

- Diagnosis: ^E.g., Venous ulcer / Dehiscd surgical wound / Traumatic tear^
- Healing Phase: ^Inflammatory / Proliferative / Maturation
- Debridement needed: ^None / Autolytic / Conservative Sharp / Mechanical / Biological

Plan

- Debridement performed: <Type: > - <Result: >
- Cleansing: <Normal saline / Prontosan / Tap water>
- Primary Dressing: <E.g., Inadine / Aquacel Ag / Hydrogel / Foam>
- Secondary Dressing/Fixation: <E.g., Zetuvit / Hypafix / Compression bandage>
- Frequency: <Review in __ days / PRN if strike-through>
- Referral/Escalation: <Wound clinic / Vascular / Not required>

*If a wound is stalled, use the assessment section to document if you've considered underlying causes (e.g., "Check HbA1c/Iron/ABI if no progress in 2 weeks").

